Rectal & Genitourinary Assessment

NURA 330 Health Assessment | Module 7 Lecture Study Guide

Exam 2 content · reviewed for the final

Lecture roadmap: Case: T.K. (BPH) and PSA → Urinary anatomy → Female and male reproductive anatomy → Rectal anatomy → Health history (urinary, anal, reproductive; present and past) → Rectal exam findings and Bristol stool chart → Female and male GU findings → Urine appearance and urine output → Urinary retention and bladder scanning → UTI assessment → Health promotion (vaccines, contraception, STI screening, cancer screening) → Lifespan → Psychosocial and SDOH → Gender-affirming care → Terminology

Exam 2 study guide topics: reproductive A&P; inclusive communication and patient-centered interviewing; subjective history (**LMP, GTPAL, STIs, contraception, urinary patterns**); lifespan (**Tanner stages and BPH**); expected vs. unexpected findings and EHR documentation. GTPAL and Tanner stages are on the study guide but not in the slides; both are covered here and tagged.

How to read this guide: Everything without a tag comes from the lecture slides or the slide notes. Anything else is tagged: STUDY GUIDE on the exam study guide but not in the slides (written from standard references); ADDED information not in the slides; CLARIFIED a slide point explained further; CORRECTED an error in the slides. Every tag is also listed in the notes at the end.

Case: T.K., 73-Year-Old Male

Presentation: Outpatient visit for urinary complaints: **increased frequency, difficulty starting the stream, dribbling**, and **nocturia** (up more than six times a night, trouble falling back asleep). Began about 3 months ago, worse over the past week. **Denies** incontinence, painful urination, or fever. No medical history, no meds, no allergies. VS: T 98.8 °F, P 80 regular, RR 22, BP 138/82, SpO_2 98% RA; BMI 24.41.

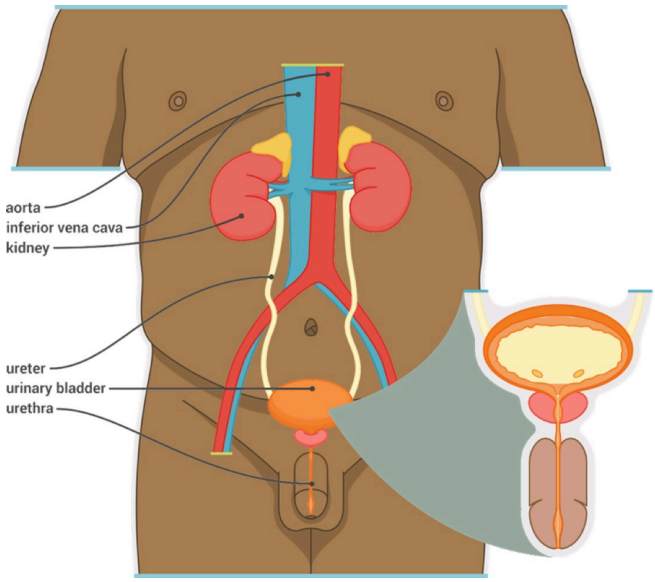
Outcome: exam normal except an **enlarged prostate**. Diagnosis: **benign prostatic hyperplasia (BPH)**. The provider orders a **PSA** and prescribes **tamsulosin 0.4 mg daily** (the alpha-blocker from Pharm), with follow-up in 2 weeks.

PSA (prostate-specific antigen)	
Purpose	Screening for early prostate cancer, or monitoring response to prostate cancer treatment
Normal	Low levels. Total PSA under about **2.5–4.0 ng/mL**; thresholds vary with age
High	**Does not automatically mean cancer.** Also raised by **BPH**, **prostatitis**, **UTIs**, and recent prostate irritation (heavy bicycling, sexual activity)
Teaching	Avoid ejaculation for **2 days** before the PSA blood test

Anatomy

Urinary System

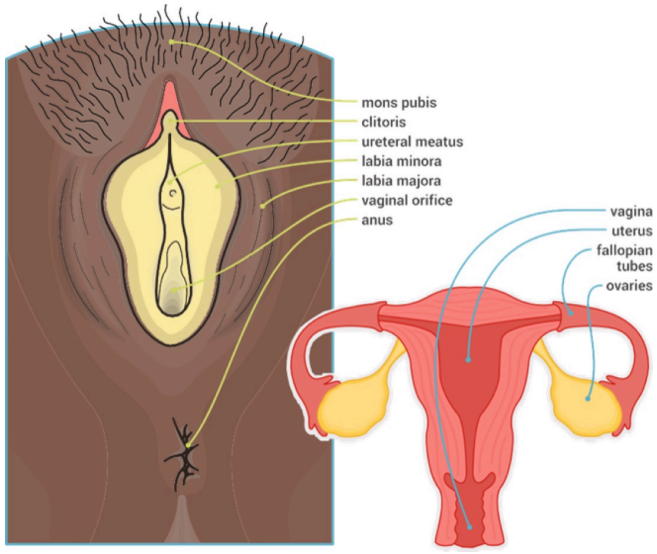
Kidneys	Two fist-sized, bean-shaped organs, below the ribcage, posterior CORRECTED (slide: "first sized"). Regulate fluid, electrolytes, and acid-base balance; filter waste (water, salt, urea) into urine; secrete **erythropoietin** (RBC production); help control BP
Ureters	Muscular tubes, 25–30 cm, moving urine to the bladder by **peristalsis**; a fold at the bladder acts as a one-way valve to prevent reflux
Bladder	Hollow, muscular, stretchy; behind the symphysis pubis. **About 300 mL creates the urge to urinate**
Urethra	Male: about **20 cm**, ends at the glans, carries urine and semen. Female: about **3–4 cm**, meatus between the clitoris and vagina. **The short female urethra raises UTI risk**



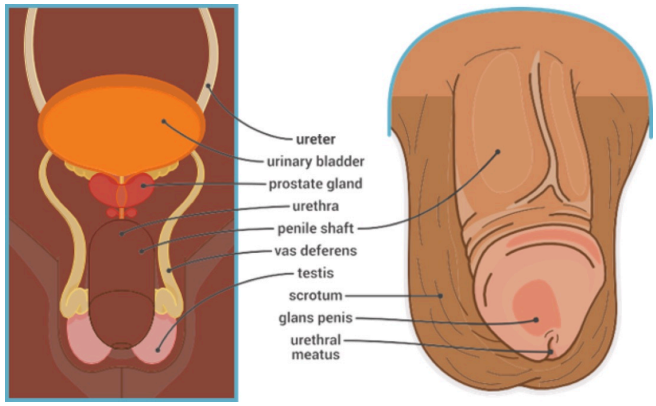
Urinary system (lecture slide)

Reproductive System

	Female	Male
External	**Vulva**: mons pubis, labia majora, labia minora, vestibule, urethral meatus, vaginal orifice	**Penis, scrotum**
Internal	**Vagina, uterus, fallopian tubes, ovaries**	**Testes, epididymis, vas deferens, ejaculatory duct, urethra, seminal vesicles, prostate gland**
Hormones ADDED	Ovaries make eggs, estrogen, progesterone	Testes make sperm and testosterone. The prostate surrounds the urethra below the bladder, which is why BPH affects urination



Female reproductive anatomy (lecture slide). **CORRECTED** *The diagram's "ureteral meatus" label should read* ***urethral meatus****.*



Male reproductive anatomy (lecture slide)

Rectal Anatomy

The rectum is the end of the sigmoid colon, with valves that control the passage of stool and gas. The **perianal area** surrounds the anal opening and sits close to the reproductive organs. Two sphincters: the **internal sphincter is involuntary**; the **external sphincter is voluntary**. The anus stays closed except when passing stool or gas.

Health History

Key principle: the reproductive history is guided by the client's **sex assigned at birth, gender identity, and current anatomy**. Ask about identity, gender-affirming treatment, and sexual practices **tactfully and nonjudgmentally**. Ask permission before sensitive questions, explain why they matter, and explain privacy and its limits.

Present Health

System	Ask about
Urinary	Trouble passing urine; **urgency**; how often you void; getting up at night (**nocturia**); **suprapubic or low back pain** (if yes, check for bladder distention); frequency; **pain with urination**; urine color, clarity, odor; **difficulty starting the stream**; change in stream force; **dribbling**; **leaking with coughing or laughing**; incomplete emptying; genital discharge (color, odor, onset); lesions, itching, genital pain; weight gain, ankle swelling, or SOB (possible kidney failure)
Anus	Hemorrhoid symptoms (itching, pain, burning); stool color changes; **blood, pus, or mucus in stool**; change in bowel habits (diarrhea, constipation, incontinence)
Reproductive	Gender identity, sex assigned at birth, gender-affirming treatment, sexual practices; how often the client has a professional genital exam

Past Health History

- **Urinary:** UTIs; personal or family history of prostate problems (BPH) or kidney disease.
- **Anus:** hemorrhoids; rectal or anal surgery.
- **Reproductive:** personal or family history of reproductive cancers; reproductive surgery; family history of cervical, ovarian, breast, prostate, testicular, or colon cancer.

Female Reproductive History

- Clients who have had a **hysterectomy no longer menstruate**. With masculinizing testosterone, **periods usually stop within 3–6 months**.
- If still menstruating: **last menstrual period (LMP)**; age at **menarche**; cycle frequency and duration; excessive bleeding, clots, cramping. Unexpected findings → referral for a gynecologic exam.
- **Obstetric history:** number of pregnancies and deliveries; miscarriages or abortions; living children; trouble getting pregnant; contraceptive use.
- **Over 40:** menopause symptoms (sweating, **hot flashes**, **vaginal dryness**); hormone replacement therapy. A total hysterectomy (uterus and ovaries) causes menopause symptoms unless on HRT.

CLARIFIED **LMP** means the **first day** of the last menstrual period. Ask before any medication, imaging, or procedure in a client who could be pregnant.

Obstetric History: GTPAL STUDY GUIDE

The slides ask about pregnancies, deliveries, miscarriages/abortions, and living children; GTPAL is the standard way to record those answers.

G: Gravida	Total pregnancies, **including a current one**
T: Term	Births at **37 weeks or later**
P: Preterm	Births from **20 weeks to 36 weeks 6 days**
A: Abortions	Pregnancies ending **before 20 weeks** (miscarriage or induced)
L: Living	Currently living children

Examples: one term birth, one miscarriage at 10 weeks, pregnant now → **G3 T1 P0 A1 L1**. Twins at 34 weeks (both living) plus one term birth → **G2 T1 P1 A0 L3** (twins = one pregnancy, one birth event, two living children).

Male Reproductive History

Lumps, bulges, swelling, or changes in the testicles or scrotum; change in size; **regular testicular self-exams**.

Physical Assessment: Expected vs. Unexpected

Anal and Perianal Area

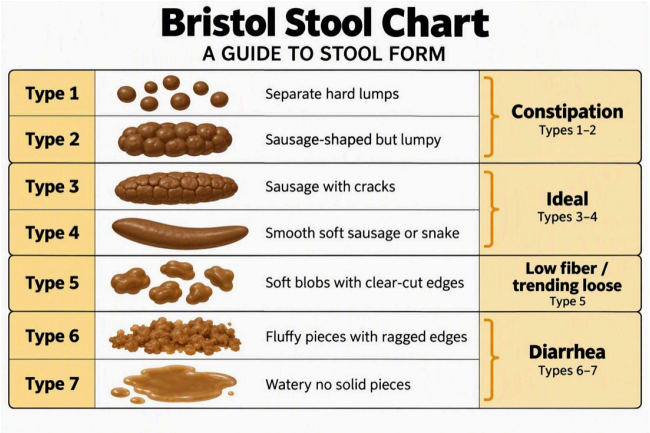
Expected	Unexpected
Intact skin; no protrusions or lesions; **darker in color** than surrounding skin; moist and hairless; **anus closed**	Inflammation, lesions, scars, **fissures**, **external hemorrhoids**, **rectal prolapse**, skin tags, bleeding, incontinence

Bristol Stool Chart

Classifies stool into **7 types** by shape and consistency to judge transit time and digestive health.

Types	Appearance	Meaning
1–2	Separate hard lumps; lumpy sausage	**Constipation**

Types	Appearance	Meaning
3–4	Sausage with cracks; smooth soft sausage	**Ideal**
5	Soft blobs with clear edges	Low fiber / trending loose
6–7	Fluffy ragged pieces; watery, no solids	**Diarrhea**



Bristol stool chart (lecture slide)

Female Genitalia

Expected	Unexpected
Even hair distribution; clear skin, appropriate color; symmetric; **urethral meatus midline, star- or slit-shaped**; clitoris round and midline. **Expected variations:** scars, atrophy	Swelling, redness; patchy or complete hair loss; lesions or ulcers; **lice**; **genital warts**; inflammation, edema; excoriation; **discolored or foul-smelling discharge**; tenderness, pain, **bruising**

Male Genitalia

Expected	Unexpected
Scrotal skin wrinkled; visible vasculature; **glans smooth**; **foreskin (prepuce) retracts** if present; **testes equal in size**	Inflammation, edema; lesions, ulcers; **nodules**; lice; **unequal or swollen testes**

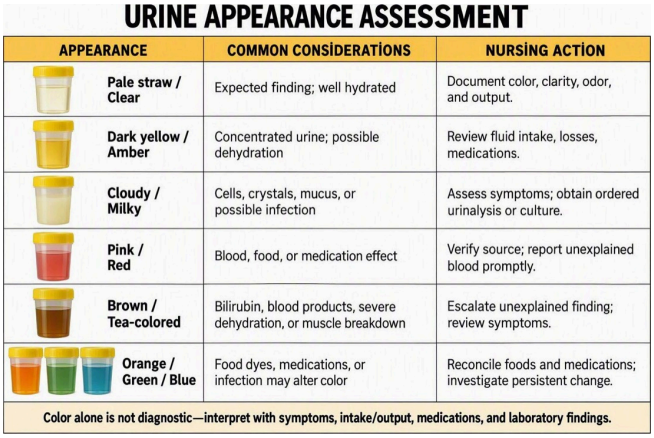
ADDED **Urgent male findings:** **sudden, severe scrotal pain and swelling** may be **testicular torsion** (an emergency needing surgery within hours). A **hard, painless testicular lump** needs prompt evaluation for cancer, which is most common in men about 15–35. A foreskin stuck in the retracted position (**paraphimosis**) is also urgent.

Urinary Assessment

Urine Appearance

At the bedside, assess **amount, color, clarity, odor**. Ask about changes from usual, symptoms, fluid intake, recent foods, and medications. **Color is a screening observation, not a diagnosis.**

Appearance	Consider	Nursing action
Pale straw / clear yellow	**Expected**; well hydrated	Document color, clarity, odor, output
Dark yellow / amber	Concentrated; possible dehydration	Review intake, losses, meds, I&O, VS, mucous membranes
Cloudy / milky	Cells, bacteria, crystals, mucus, contamination, possible infection	Don't diagnose by look. Ask about dysuria, urgency, frequency, suprapubic or flank pain, fever, **new confusion**; obtain ordered UA/culture with correct technique
Pink / red	Blood, foods, medications	Rule out menstrual contamination; **report unexplained visible blood promptly, even without pain**
Brown / tea-colored	Bilirubin, blood products, severe dehydration, **muscle breakdown**	**Escalate**; assess for jaundice, muscle pain or injury, weakness, low output, med changes
Orange / green / blue	Medications, dyes, foods, sometimes infection	Reconcile meds and foods; investigate persistent change



URINE APPEARANCE ASSESSMENT

APPEARANCE	COMMON CONSIDERATIONS	NURSING ACTION
Pale straw / Clear	Expected finding; well hydrated	Document color, clarity, odor, and output.
Dark yellow / Amber	Concentrated urine; possible dehydration	Review fluid intake, losses, medications.
Cloudy / Milky	Cells, crystals, mucus, or possible infection	Assess symptoms; obtain ordered urinalysis or culture.
Pink / Red	Blood, food, or medication effect	Verify source; report unexplained blood promptly.
Brown / Tea-colored	Bilirubin, blood products, severe dehydration, or muscle breakdown	Escalate unexplained finding; review symptoms.
Orange / Green / Blue	Food dyes, medications, or infection may alter color	Reconcile foods and medications; investigate persistent change.

Color alone is not diagnostic—interpret with symptoms, intake/output, medications, and laboratory findings.

Urine appearance assessment (lecture slide)

Urine Output

Urine output (mL/kg/hr) = Total urine volume (mL) ÷ [Body weight (kg) × Collection time (hours)]

ADDED **Normal and example:** adult output of at least **0.5 mL/kg/hr** (often rounded to about 30 mL/hr) is generally adequate; below that is **oliguria**. Example: 70-kg adult, 280 mL over 8 hours → 280 ÷ (70 × 8) = **0.5 mL/kg/hr**.

Urinary Retention

Subjective	Reports difficulty voiding. Ask about pain, burning, hesitancy, urgency, a full feeling after voiding, kidney history; in males, prostate symptoms (hesitancy, dribbling)
Inspect	**Suprapubic area for distention**. Causes: obstruction, weak bladder muscles, medications, neurologic conditions
Palpate	A distended bladder is felt **above the symphysis pubis, midline**, as a **smooth, rounded mass**; note tenderness
Bladder scanner	Measures urine volume. Client **supine**; gel **2.5–4 cm above the symphysis pubis**; probe pointed down. For **post-void residual (PVR)**, scan within **10 minutes** of voiding. **Expected PVR 50–100 mL; over 100 mL → notify the provider**
Other data	Compare **intake and output**; intake much greater than output → notify provider

Promote voiding	Position: males standing or using a urinal; females sitting or squatting (ensure safe transfer). **Sound of running water** or **hand in warm water**

CLARIFIED The slide also lists "recent urine voids (30–75 mL/hour)" without explanation. Small, frequent voids can be a sign of overflow from a bladder that isn't emptying, which is why a bladder scan follows.

Urinary Tract Infection

Subjective	Assessment	Interventions
Urgency, frequency, burning with urination, blood in urine, foul odor	Urine color, smell, amount; **urinalysis** and **urine culture**	Antibiotics, pain medication, prevention teaching

Blood in the urine (**hematuria**) can come from menstruation or a kidney problem (e.g., infection). **Stress incontinence** = leaking with coughing or sneezing (after childbirth or from neuromuscular problems).

Health Promotion

Vaccines

- **HPV:** linked to cervical, anal, and penile cancers; spread by skin-to-skin contact; often no symptoms. 2–3 doses over 6 months; recommended starting at **age 9 for all genders**, catch-up through **26**, some adults 27–45 after discussion. **Not given in pregnancy.** Best before sexual activity begins.
- **Hepatitis B:** spread by blood, semen, body fluids; for high-risk groups (healthcare workers, blood exposure, travel to endemic areas, chronic liver disease, HIV).

Contraception

Natural	No meds or devices: withdrawal, fertility tracking, periodic abstinence
Barrier	Condoms, diaphragms, sponges. **Condoms prevent both STIs and pregnancy**
Pharmacologic	Oral or injectable hormones, spermicides. **Oral contraceptives do NOT prevent STIs**
Implanted	**IUD**: long-lasting, reversible, 3–10 years, placed in the office

Surgical	Permanent: **vasectomy, tubal ligation**

Counsel based on the client's lifestyle, preferences, risk, culture, and beliefs, and teach correct use.

STI Prevention and Screening

- Do a respectful, nonjudgmental **behavioral risk assessment**. Teach transmission and risk reduction. **New condom for each sexual encounter.**
- **HIV:** CDC recommends screening ages **15–75** and in **pregnancy**; **annually if high risk**, once if low risk.
- **Chlamydia and gonorrhea:** sexually active clients **under 25**, and those with multiple partners or possible exposure.

Cancer Screening

Screening	Recommendation (slide notes)
Colorectal	Start at **age 45** (average risk); ask family history and the relative's age at diagnosis. Options: **FOBT or FIT yearly**; FIT-DNA every 3 years; flexible sigmoidoscopy every 5 years (or every 10 with yearly FIT); **colonoscopy every 10 years**
Cervical (Pap)	**Ages 21–29: Pap every 3 years. Ages 30–65:** Pap + HPV co-test every 5 years, Pap alone every 3 years, or HPV alone every 5 years. May stop after 65 with normal results in the past 10 years, or after hysterectomy with cervix removed
Prostate	PSA and DRE (DRE by provider) are an **individual decision**. ACS: offer at **50** for average risk (life expectancy ≥10 years); **40–45 for high risk**. PSA <2.5 → every 2 years; >2.5 → yearly. Prostate and genital exam yearly
Testicular	Exam with the annual physical; **monthly testicular self-exam**

Lifespan Considerations

Infants & children	Adolescents & adults	Older adults
Caregiver history plus age-appropriate language; children may not pinpoint symptoms. Assess **toilet training, enuresis** (bedwetting), constipation or stool holding, hydration, hygiene, congenital history, UTI signs	**Private, inclusive** assessment of urinary, bowel, sexual, and reproductive health; screen for infection, pregnancy, STI risk. Consider menstruation, pregnancy/postpartum changes, pelvic floor symptoms, meds, occupation, **colorectal screening at 45**	Urgency, **retention**, incontinence, constipation or fecal leakage, **BPH**, pelvic organ prolapse, mobility, cognition, meds, toilet access. **Acute confusion may signal infection. Incontinence is common but not normal aging.** Individualize screening after 75

BPH in Older Adults

As in T.K.'s case: **frequency, hesitancy (trouble starting), weak stream, dribbling, nocturia, incomplete emptying**, possible retention. The enlarged prostate squeezes the urethra. The provider confirms it by DRE; PSA may be ordered; tamsulosin relaxes the prostate.

Tanner Stages STUDY GUIDE

A **5-stage scale** of pubertal development: **stage 1 = prepubertal**, **stage 5 = adult**. Rated for breasts and pubic hair (females) and genitals and pubic hair (males). **First sign of puberty:** breast budding in girls (about 8–13); testicular enlargement in boys (about 9–14). Puberty before 8 (girls) or 9 (boys), or none by about 13/14, needs evaluation.

Stage	Breasts	Male genitals	Pubic hair
1	Prepubertal	Childhood size	None
2	Breast bud	Testes and scrotum enlarge	Sparse, fine, straight
3	Breast and areola enlarge	Penis lengthens	Darker, coarser, curlier
4	Areola forms secondary mound	Penis widens; glans develops	Adult type, not on thighs
5	Adult	Adult	Adult, spreads to inner thighs

Psychosocial Assessment & SDOH

Approach: **Ask permission** before sensitive questions, **explain why they matter**, and **explain privacy and confidentiality limits**. Offer a **chaperone** and preserve privacy.

Area	Explore
Emotional response & dignity	Embarrassment, anxiety, depression, body image, prior trauma, comfort with exams
Urinary symptoms & daily life	Urgency, frequency, pain, leakage, odor; effect on sleep, work, travel, exercise, independence
Sexual & reproductive health	**Inclusive, nonjudgmental** questions about function, partners and practices, STI concerns, fertility goals, contraception, identity, name, pronouns
Bowel & rectal concerns	Distress about bleeding, pain, constipation or diarrhea, fecal leakage, odor, ostomy care, fear of cancer or screening
Self-care & access	Private toilet access, hygiene, meds and supplies, diet and hydration, health literacy, transportation, insurance, language
Coping, support & safety	Coping, supports, cultural and spiritual beliefs, caregiver burden, **coercion or violence**, referrals to social work or behavioral health

SDOH (both GU and rectal slides): economic (cost of visits, tests, bowel prep, meds, contraception, continence supplies; time off work; high-fiber food); **education/health literacy** (knowing symptoms, screening, prep, when to seek urgent care); **healthcare access** (primary care, urology, nephrology, GI, prenatal and STI services, transportation, insurance, language); **environment** (housing, clean water, sanitation, private and accessible toilets, restroom access at work, toxins); **social context** (**stigma** about GU or bowel symptoms, infertility, sexual or gender identity; partner violence; support).

Gender-Affirming Care

- About 1.4 million U.S. adults identify as transgender (gender identity differs from sex assigned at birth). **Use the client's stated name, pronouns, and terms.**
- **Assess current anatomy, treatments, goals, screening needs, and preferences. Do not infer them from identity.** Care follows the person's goals.
- **Feminizing hormones:** estrogen and antiandrogens (including **spironolactone**): less muscle and body/facial hair, breast development, lower libido, fat redistribution; voice

usually unchanged. Spironolactone can cause **hyperkalemia**.

- **Masculinizing hormones:** testosterone: deeper voice, more facial and body hair, scalp hair loss, acne, more muscle, weight gain, clitoral enlargement, vaginal atrophy; **periods usually stop in 3–6 months**.
- Hormone therapy risks include hormone-sensitive cancers; some changes are permanent or affect fertility, so discuss **sperm or egg preservation** before treatment or surgery.
- **Surgeries:** transgender women may have breast augmentation, orchiectomy, vaginoplasty. Transgender men may have chest reconstruction (double mastectomy), metoidioplasty or phalloplasty, hysterectomy, oophorectomy.
- Screening follows **anatomy present** (e.g., a transgender man with a cervix still needs cervical screening). ADDED

Medical Terminology

Genitourinary	Rectal & bowel
Dysuria: painful or difficult urination **Hematuria:** blood in urine **Frequency:** urinating more often than usual **Urgency:** sudden, compelling need to urinate **Nocturia:** waking at night to urinate **Oliguria:** abnormally low output **Polyuria:** excessive output **Urinary retention:** can't fully empty bladder **Incontinence:** involuntary leakage	**Hematochezia:** bright red blood with stool **Melena:** black, tarry stool (digested blood) **Constipation:** infrequent or difficult stool **Diarrhea:** frequent loose or watery stool **Fecal incontinence:** accidental passage of stool or mucus **Rectal prolapse:** rectal tissue protrudes through the anus

Documentation ADDED

The slides assign GU and rectal EHR documentation as practice but do not give examples. Use objective terms, separate reported from observed, and record LMP, obstetric history, and chaperone and consent.

- **Normal:** "External genitalia without lesions, erythema, edema, or discharge. Urethral meatus midline. Perianal skin intact, no hemorrhoids or fissures. Voiding clear yellow urine without difficulty; denies dysuria, frequency, urgency. Last BM today, Bristol type 4."
- **Abnormal:** "Client reports difficulty starting stream and dribbling × 3 months; nocturia 6×/night. Suprapubic area distended, smooth rounded mass palpated above symphysis pubis. Bladder scan PVR 350 mL. Provider notified."

Quick Review

Topic	Key points
Bladder	Urge at about 300 mL; distended bladder = smooth round mass above symphysis pubis
Female urethra	3–4 cm (short) → more UTIs
Sphincters	Internal involuntary, external voluntary
PVR	Scan within 10 min of voiding; expected 50–100 mL; >100 mL notify
Urine color	Pale straw = expected; red or brown unexplained = report promptly; color is not a diagnosis
Output	mL ÷ (kg × hours); ≥0.5 mL/kg/hr adequate
UTI	Urgency, frequency, burning, hematuria, odor; UA and culture; older adults may only show confusion
Bristol	1–2 constipation; 3–4 ideal; 6–7 diarrhea
BPH	Hesitancy, weak stream, dribbling, frequency, nocturia; PSA can be high without cancer
Screening	Colorectal at 45; Pap 21–29 every 3 yrs; HPV vaccine from 9; HIV 15–75; chlamydia/gonorrhea under 25; monthly testicular self-exam
Inclusive care	Ask permission; anatomy, not identity, guides screening; stated name and pronouns; chaperone
Melena vs. hematochezia	Black tarry (upper GI) vs. bright red

Self-Check ADDED

1. A bladder scan 5 minutes after voiding shows 220 mL. Action? *Answer: PVR over 100 mL; notify the provider.*
2. Why are UTIs more common in females? *Answer: The urethra is short (3–4 cm).*
3. T.K.'s PSA comes back elevated. Does he have cancer? *Answer: Not necessarily; BPH, prostatitis, UTI, or recent activity can raise PSA.*
4. An 82-year-old has new confusion and cloudy, foul-smelling urine. What should the nurse suspect? *Answer: UTI; report it and obtain the ordered UA/culture.*

5. A client reports black, tarry stools. Term? *Answer: Melena.*
6. A transgender man on testosterone asks if he still needs Pap tests. Response? *Answer: Screening follows anatomy; if he has a cervix, yes.*
7. Which contraceptive method also prevents STIs? *Answer: Condoms.*
8. An 80-kg client voided 240 mL over 6 hours. Adequate? *Answer: 240 ÷ (80 × 6) = 0.5 mL/kg/hr; borderline adequate.*

Notes on Sources, Additions & Clarifications

Corrections:

- The kidney slide says "first sized"; it should be **fist-sized**.
- The female anatomy diagram labels the "ureteral meatus." The opening is the **urethral meatus** (the ureters connect kidneys to bladder and have no external opening).
- The slide notes refer to "benign prostatic hypertrophy"; the slide and the standard term is benign prostatic **hyperplasia**.
- The gender-affirming care notes list "deeper voice, increased body hair growth" among spironolactone's side effects. Spironolactone blocks testosterone, so those effects would be unexpected; that list looks partly copied from testosterone. The hyperkalemia, breast enlargement, and erectile dysfunction it lists are consistent with ATI.

From the Exam 2 study guide (not in the slides): GTPAL and Tanner stages. Written from standard nursing references; check the specifics against ATI Health Assess 3.0 "Rectum and Genitourinary."

Added information: hormone roles in the anatomy table; urgent male findings (torsion, testicular mass, paraphimosis); the urine output norm (0.5 mL/kg/hr) and worked example; the "screening follows anatomy" point; documentation examples; the self-check questions.

Clarified: LMP means the first day of the last period; a possible meaning for the slide's "recent urine voids (30–75 mL/hour)" line.